Zoledronic acid · adjuvant breast treatment

Zoledronic acid by intravenous (IV) infusion once every six months for up to five years, unless progression or unacceptable toxicity changes the plan. The usual source dose is 4 mg, with renal review before prescribing.

Adjuvant bone-modifying treatment · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Jaw / mouth

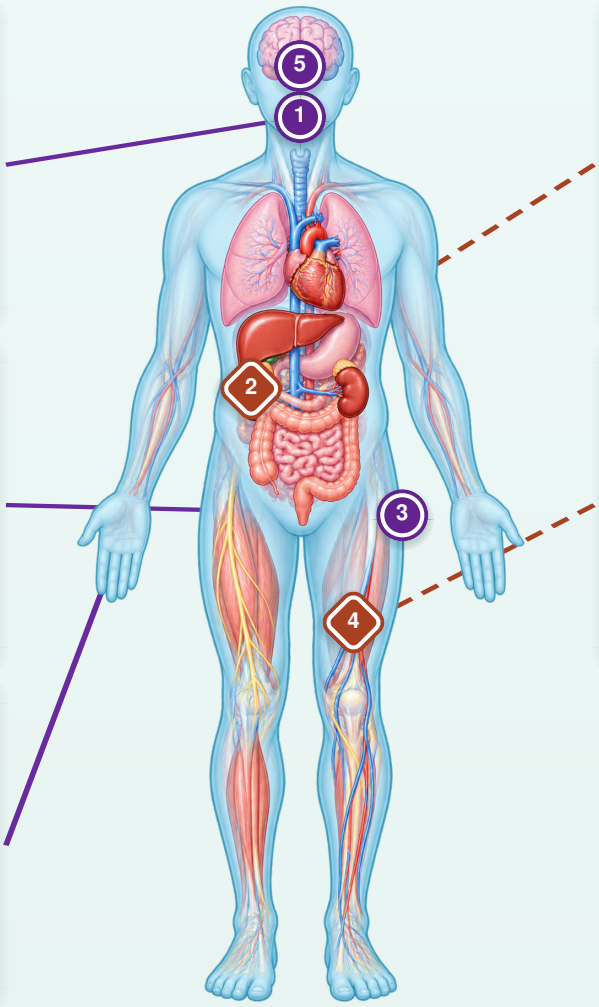
Zoledronic acid

Report jaw pain, exposed bone or a wound that does not heal.

2 Kidneys / urine

Zoledronic acid

Report reduced urine or marked swelling.



3 Muscles / calcium

Zoledronic acid

Tingling, cramps or spasms may need urgent blood tests.

4 Thigh / groin

Zoledronic acid

Report new unexplained thigh or groin pain.

5 Eyes

Zoledronic acid

Painful red eyes or vision changes need prompt review.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Report jaw pain, exposed bone or a non-healing mouth wound, reduced urine, tingling or muscle spasms, painful red eyes, or new thigh/groin pain. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

YOUR PRACTICAL TREATMENT PLAN

Know the next step.

Taking treatment

Zoledronic acid by IV infusion once every six months for up to five years, unless progression or unacceptable toxicity changes the plan. The usual source dose is 4 mg, with renal review before prescribing.

The infusion itself runs over at least 15 minutes. Preparation, blood review and observation make the visit longer. A port or take-home infusion pump is not routinely required for this medicine alone.

Checks and follow-up

Before starting: renal profile, magnesium, calcium and phosphate. Check creatinine before every infusion; compare it with baseline as well as reviewing creatinine clearance. Correct low calcium and review vitamin D. Complete a dental assessment and preventive care before treatment where appropriate. Report dental symptoms; discuss invasive dental work with oncology and dental teams. Hydration is individualised, especially with heart failure.

The whole-treatment map shows the checks for this exact treatment setting.

Support between reviews

Calcium and vitamin D

Follow the prescribed daily supplements; the team checks calcium and adjusts them if it is high or low.

Dental care

Keep routine dental care and tell the dentist about treatment. Report jaw pain or a wound that does not heal.

Flu-like symptoms

Aches or fever can occur after infusion. Use the agreed symptom plan; seek advice if severe or persistent.

Hydration

Follow the team’s fluid advice; heart or kidney problems may change it.

Oncology daytime: Insert numbers here. **Out of hours:** Insert numbers here. Follow your own hospital’s urgent-care plan. Emergency: 112 / 999.

Your complete treatment reference

https://sactcheck.com/patient/00545/

Public information only. No personal information is encoded.